

Ethics & The Law in Psychiatry 11/07/2025

Ethics in Psychiatry Lecture Objectives

By the conclusion of this one-hour lecture, students will be able to

- A. Identify the major medical ethical principles pertinent to medical/psychiatric care. (Including autonomy and Psychiatric Advance Directives)
- B. Understand what is Paternalism
- C. Understand the use of power in medicine.
- D. Understand what are the types of Boundary Violations
 - 1. Understand what is a therapeutic frame
 - 2. Understand what is a boundary crossing?
 - 3. Recognize Red Flag Warnings of CA Medical Board
 - 4. Understand “General Truths”
- E. Discuss the essential aspects of confidentiality, and informed consent.

The Law in Psychiatry: Lecture Objectives

By the conclusion of this one-hour lecture, students will be able to:

- A. Identify important factors pertaining to malpractice, including why psychiatrists are sued.
- B. Understand the two Tarasoff Decisions
- C. Understand what is privilege and the various levels of privilege.
- D. Discuss important CA civil commitment statutes. (e.g., 5150 and 5250)
- E. Understand important legal cases in “Right to receive”, and “Right to Refuse Treatment”.
- F. Understand important Penal Code Statutes as well as Supreme Court Cases regarding Competency to Stand Trial (P.C. 1370) and Not Guilty by Reason of Insanity (NGI: P.C. 1026) as well as the M’Naghten Doctrine
- Differences between treatment in a state hospital vs. a prison/jail
- Statistics regarding patients in prisons with mental health issues
- H. Understand meaning of legal terms: Mens Rea and Actus Rea
- I. Understand what is meant by testamentary capacity.

Ethics & Law in Psychiatry — Summary

I. Core Medical Ethics in Psychiatry

Psychiatric care relies heavily on ethical principles because of issues of autonomy, capacity, coercion, and confidentiality.

A. Major Ethical Principles

1. Autonomy

- A patient's right to make decisions about their own care.
- Includes **Psychiatric Advance Directives**, which allow patients to specify treatment preferences for future episodes of incapacity.

2. Beneficence – Act in the patient's best interest.

3. Non-maleficence – Avoid causing harm.

4. Justice – Fair and equal treatment.

B. Paternalism

- When a physician overrides a patient's wishes to protect their health.
- Acceptable only when the patient **lacks decision-making capacity** or there is **imminent risk** to self/others.

C. Power in Medicine

- Physicians inherently hold structural and interpersonal power.
- Requires awareness to avoid coercion—especially crucial in involuntary psychiatric treatment.

II. Boundary Issues in Psychiatry

Psychiatry has heightened risks because of prolonged, intimate, and emotionally charged therapeutic relationships.

A. Types of Boundary Violations

1. Therapeutic Frame

- The consistent structure of therapy: time, place, payment, roles, limits.

2. Boundary Crossing

- A minor deviation from the frame (e.g., accepting a small culturally appropriate gift).
- May be acceptable if *therapeutically justified*.

3. Boundary Violation

- Harmful, exploitative breach (e.g., dual relationships, financial exploitation, sexual contact).

4. Red Flag Warnings (CA Medical Board)

- Excessive self-disclosure by the physician
- Role reversals
- Secret relationships
- Dual relationships
- Physical contact of unclear purpose
- Special treatment or favoritism

5. General Truths

- The burden of maintaining boundaries lies entirely with the physician.

- All sexual contact with patients is unethical and illegal.
 - Documentation and supervision reduce risk.
-

III. Confidentiality & Informed Consent

- **Confidentiality** is fundamental; limits include:
 - danger to self or others
 - suspected abuse/neglect
 - court orders
 - **Informed Consent** requires:
 - explanation of nature of treatment
 - risks and benefits
 - alternatives
 - voluntary participation
 - capacity to decide
-

IV. Psychiatry & the Law

A. Malpractice in Psychiatry

Psychiatrists are most often sued for:

- Failure to prevent suicide or harm to others
- Improper diagnosis or medication management
- Boundary violations
- Breach of confidentiality

Malpractice requires **duty** → **breach** → **harm** → **causation**.

B. Tarasoff Decisions

1. Tarasoff I (Duty to Warn)

- Clinicians must warn identifiable victims of credible threats.

2. Tarasoff II (Duty to Protect)

- Broader duty: may involve hospitalization, warning police, or notifying victim.
-

C. Privilege

- Legal protection preventing disclosure of therapy discussions in court.
 - **Levels:**
 - *Absolute* (rare)
 - *Qualified* (often psychiatrist-patient)
 - *Limited by exceptions* (danger, court orders, child abuse)
-

D. Civil Commitment (California)

Key holds:

- **5150** – 72-hour involuntary hold for danger to self, others, or grave disability.
 - **5250** – 14-day hold following 5150 if criteria persist.
-

E. Right to Receive & Right to Refuse Treatment

- Patients have rights to reasonable care in institutional settings.
- They may refuse treatment unless:

- declared incompetent
 - in emergencies
 - specific legal criteria for involuntary medication are met.
-

F. Criminal Law Concepts in Psychiatry

1. Competency to Stand Trial (PC 1370)

- Defendant must understand the proceedings and assist in their defense.
- If incompetent: treatment in state hospital until competency restored.

2. Not Guilty by Reason of Insanity (NGI, PC 1026)

- Based on **M’Naghten Rule**:
 - Defendant lacked understanding of the nature or wrongfulness of the act.

3. Treatment in State Hospital vs Prison

- State hospitals focus on mental health rehabilitation; prisons focus on security.
 - A large percentage of incarcerated individuals have mental illness.
-

G. Mens Rea & Actus Rea

- **Mens rea** – “guilty mind” (intent).
 - **Actus rea** – “guilty act” (behavior).
 - Both required for most crimes.
-

H. Testamentary Capacity

A person must understand:

1. Nature of making a will
2. Their assets
3. Their beneficiaries
4. The plan for distributing property